

NIW Petition Narrative — I-140 Cover Letter Draft

Manasa Jampani — EB-2 National Interest Waiver

Note: This is a draft narrative for attorney review. Have your immigration attorney finalize this before filing.

DRAFT I-140 COVER LETTER

[Date]

U.S. Citizenship and Immigration Services
[Service Center Address]

RE: Form I-140 Immigrant Petition for Alien Workers
Preference Category: EB-2 National Interest Waiver
Petitioner: Manasa Jampani
Basis: Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)

Dear Officer:

This letter is submitted on behalf of Manasa Jampani in support of her Form I-140 petition for classification as an EB-2 immigrant with a National Interest Waiver of the job offer and labor certification requirements pursuant to INA § 203(b)(2)(B)(i) and the standards established in Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016).

Ms. Jampani is the Co-Founder and CEO of Ardia Health Labs, a Delaware LLC incorporated in January 2026 and headquartered in Argyle, Texas. She holds a degree in [degree, institution] and has 10+ years of IT experience, including 5+ years in healthcare IT at Interwell Health, United HealthGroup, and ECFMG.

As set forth below and supported by the accompanying evidence, all three prongs of the Dhanasar framework are satisfied.

I. PRONG 1: THE PROPOSED ENDEAVOR HAS SUBSTANTIAL MERIT AND NATIONAL IMPORTANCE

Ms. Jampani's proposed endeavor is the co-development and commercialization of AI-native revenue cycle management infrastructure for independent clinical laboratories, specifically designed to recover preventable claim denials in the molecular diagnostics, pharmacogenomics, and next-generation sequencing billing domains.

The National Problem

The United States independent clinical laboratory sector faces an existential financial crisis driven by a structural claim denial problem. As documented by the following sources submitted with this petition:

- A 2025 Georgetown University study published in JAMA Network Open, analyzing 29,919 next-generation sequencing claims, documented a 23.3% overall denial rate — rising to 27.4% following CMS national coverage determination changes — and found that independent laboratories face 2.76 times higher odds of claim denial compared to hospital-based laboratories after multivariate statistical adjustment. (Tab [A])
- The 2024 XiFin Payor Denial Impact Report, analyzing more than 20 million laboratory claims, documented a 35.3% denial rate for molecular diagnostic claims — the highest denial rate across all healthcare specialties. (Tab [B])
- The 2023 Frontiers in Pharmacology study found that only 46% of pharmacogenomics claims are reimbursed, despite robust evidence of clinical utility and FDA pharmacogenomic labeling for 200+ drugs. (Tab [C])
- The 2024 HFMA/LigoLab Laboratory Revenue Recovery Report documented that 65% of denied laboratory claims are never appealed, despite documented appeal win rates of 50–80.7%. (Tab [D])

These statistics collectively describe an estimated \$10–12 billion in annual preventable revenue loss to the US independent laboratory sector.

The consequences of this crisis extend beyond laboratory finances to healthcare access. The 2024 American Clinical Laboratory Association survey found that more than 50% of independent laboratory directors reported considering consolidation or sale if PAMA 2027 reimbursement cuts take effect. (Tab [E]) Independent laboratories serve as the primary diagnostic infrastructure for rural communities, addiction medicine clinics, and specialty practices that do not have access to hospital-based laboratory services. The Protected Access to Medication Act (PAMA) projects cumulative Medicare reimbursement cuts of up to 45% for clinical laboratory tests by 2029, compounding the financial pressure from the existing denial burden. (Tab [F])

The national importance of Ms. Jampani's endeavor is demonstrated by: - The scale of the economic problem (\$10–12 billion annual preventable loss) - The number of institutions at risk (7,000+ CLIA-certified independent laboratories) - The populations served by these institutions (rural communities, underserved populations, addiction medicine patients) - The

regulatory urgency (PAMA 2027 timeline) - The AI governance significance (native compliance with Texas SB 1188 and TRAIGA, two of the most comprehensive state AI governance laws in the United States)

II. PRONG 2: MS. JAMPANI IS WELL POSITIONED TO ADVANCE THE PROPOSED ENDEAVOR

Ms. Jampani is uniquely positioned to advance the mission of Ardia Health Labs for reasons that cannot be satisfied by any other available individual.

Payer-Side Clinical Operations Experience

Ms. Jampani has direct healthcare IT experience inside payer and payer-adjacent organizations — United HealthGroup, one of the largest health insurance companies in the United States (Fortune 7), and Interwell Health, a specialized renal care management organization that operates in the payer-adjacent clinical operations space. This experience gave Ms. Jampani direct operational visibility into how payer clinical operations teams configure, maintain, and optimize the AI-assisted claim denial systems that independent laboratories face.

Specifically, Ms. Jampani's experience provides operational knowledge of:

1. The automated medical necessity screening systems that generate the initial denial layer for molecular diagnostic claims
2. The coding compliance gatekeeping systems that validate CPT/PLA code assignments and diagnosis code linkage before payment
3. The prior authorization AI decision engine configuration and clinical criteria weighting that governs coverage determinations for high-value molecular tests
4. The predictive claim prioritization systems that deprioritize claims the payer predicts are at elevated denial risk

This knowledge is not available from public payer documentation, from reverse-engineering denial outcomes, or from any role other than direct employment within payer clinical operations. It is the single most valuable competitive intelligence available to a provider-side AI developer — and it is Ms. Jampani's unique professional asset.

Publication Record — Paper 4

Ms. Jampani is the sole author of a forthcoming paper titled "Inside the Payer AI Black Box: Operational Analysis of Clinical AI Denial Systems in Independent Laboratory Billing and Countermeasure Architecture," targeted for submission to NEJM AI. This paper provides an insider analysis of payer AI denial logic that no other researcher can provide from the same

vantage point. It represents a direct scholarly contribution to the field and establishes her as a recognized expert authority.

Co-Authorship on Papers 2 and 3

Ms. Jampani is a co-author on two additional papers: - “The Independent Laboratory Billing Crisis: AI-Addressable Revenue Loss in Molecular Diagnostics and Pharmacogenomics — A Systematic Review” (with Rambabu Vadlamudi) - “Compliance-by-Design: AI Governance Frameworks Under Texas SB 1188 and TRAIGA for Healthcare Revenue Cycle AI Systems” (with Rambabu Vadlamudi)

Platform Status

As of March 2026, the Ardia Health Labs platform architecture is 100% complete. The company is pursuing its first pilot program and is targeting \$3.5 million in seed financing at an \$18 million valuation cap. Ms. Jampani’s continued leadership as CEO is essential to this commercialization phase.

III. PRONG 3: IT WOULD BE BENEFICIAL TO THE UNITED STATES TO WAIVE THE LABOR CERTIFICATION REQUIREMENT

The labor certification process is designed to protect US workers from displacement by foreign workers who can fill positions that qualified US workers are available and willing to fill. This purpose is not served by requiring Ms. Jampani to complete PERM labor certification, for the following reasons.

First, Ms. Jampani’s role as Co-Founder and CEO of Ardia Health Labs is not a position that can be filled through the PERM recruitment process. Her equity interest in and co-founding relationship with the company are not transferable to a substitute candidate. A US worker hired into a CEO role at Ardia Health Labs would not have Ms. Jampani’s payer-side clinical operations experience, her co-founder relationship with Ram Vadlamudi, or the specific insider knowledge of payer denial systems that underpins the company’s competitive advantage.

Second, the PAMA 2027 reimbursement timeline creates urgency that the PERM process — which typically takes 12–18 months to complete — cannot accommodate. Independent laboratories face the beginning of PAMA cuts in 2027. AI-driven claim recovery technology deployed before this deadline has maximum national impact. A multi-year PERM delay would materially impair Ardia Health Labs’ ability to serve independent laboratories in the window of greatest national need.

Third, no comparable platform exists. Ardia Health Labs is the only AI-native RCM platform specifically architected for independent laboratory

molecular diagnostic billing with payer-side countermeasure intelligence. Ms. Jampani's continued contribution to this unique national asset serves the US interest in a manner that no substitute can replicate.

Fourth, the Texas AI governance leadership that Ardia Health Labs demonstrates — building the first healthcare revenue AI system in compliance with Texas SB 1188 and TRAIGA from inception — contributes to the development of responsible AI governance practices in the US healthcare system. This contribution has national regulatory significance that extends beyond Ardia's immediate commercial mission.

IV. CONCLUSION

All three prongs of the Matter of Dhanasar framework are satisfied for Manasa Jampani's EB-2 National Interest Waiver petition:

1. Her proposed endeavor — AI-native RCM infrastructure for independent laboratory claim recovery — has substantial merit in economic, clinical, technical, and regulatory terms, and is of national importance given the \$10-12 billion annual problem scale, 7,000+ laboratories at risk, and rural/underserved community access implications.
2. She is uniquely and exceptionally well positioned to advance this endeavor, by virtue of her payer-side clinical operations experience at United HealthGroup and Interwell Health — knowledge that is categorically unavailable to any other candidate — combined with her CEO leadership, publication contributions, and co-founder status.
3. It is beneficial to the United States to waive the labor certification requirement: her role cannot be filled through PERM recruitment, the national need is time-sensitive, no comparable platform exists, and her continued contribution advances US AI governance leadership in the healthcare domain.

We respectfully request that USCIS approve this petition.

Respectfully submitted,

[Immigration Attorney Name]

[Law Firm]

[Bar Number]

Attorney for Manasa Jampani

Supporting Documents Checklist

Required for Filing: - ☐ Form I-140 (signed, complete) - ☐ Filing fee (current amount: check USCIS.gov) - ☐ Copy of passport biodata page - ☐ Copy of current visa status (I-94 printout) - ☐ Educational credentials (degree certificates + transcripts) - ☐ Evidence of advanced degree or exceptional ability

EB-2 NIW Evidence Exhibits: - ☐ Tab A: JAMA Network Open 2025 (Georgetown NGS study) — print full paper - ☐ Tab B: XiFin 2024 Payor Denial Impact Report — print or obtain official copy - ☐ Tab C: Frontiers in Pharmacology 2023 — print full paper - ☐ Tab D: HFMA/LigoLab 2024 Laboratory Revenue Recovery Report - ☐ Tab E: ACLA 2024 Member Survey - ☐ Tab F: CMS PAMA reimbursement schedule documentation - ☐ Tab G: Texas SB 1188 text - ☐ Tab H: Texas TRAIGA (HB 149) text - ☐ Tab I: Ardia Health Labs White Paper (March 2026) - ☐ Tab J: Ardia Health Labs Pitch Deck v2 (2026) - ☐ Tab K: Employment verification letters from Interwell Health + United HealthGroup + ECFMG - ☐ Tab L: Paper 4 manuscript (or arXiv/medRxiv preprint once submitted) - ☐ Tab M: Papers 2 and 3 manuscripts (or preprints once submitted) - ☐ Tab N: Expert witness letters (2+ preferred) - ☐ Tab O: LinkedIn profile printout + article list (for published material evidence) - ☐ Tab P: Company formation documents (Delaware LLC certificate, January 2026)